BASELINE → TREATMENT → FOLLOW-UP

# When checks happen

Milestones are not equal time intervals. Your team books and reviews tests. Blood counts are checked on the day of each weekly dose. Kidney and liver tests follow the weekly regimen and prescribed plan. The team reviews the lowest counts after earlier doses before any increase, alongside bowel, nerve, infection and medicine checks.

| Check | Before starting | Early treatment | Ongoing treatment | Dose change / pause | Next decision |
|---|---|---|---|---|---|
| **Blood / kidney / liver** | Baseline blood and organ tests | Counts before every weekly dose | Weekly treatment review | Review recovery and current dose | Follow unresolved abnormalities |
| **Symptoms / medicines** | Bowel / nerve / infection review | Report early symptoms and support problems | Review all medicines and side effects | Do not restart or escalate yourself | Agree continuation and response assessment |
| **Special schedule check** | Confirm weekly capsule plan | Review nadirs after doses 1–3 | Dose increases only after team review | Keep previous dose / nadir history | Personal plan; no booked dates implied |

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00259 v6 · official regimen · HPRA Vinorelbine Accord soft capsules SmPC, 9 April 2025

THE WHOLE TREATMENT JOURNEY

# Vinorelbine capsules · non-small-cell lung cancer: treatment and checks

### Non-small-cell lung cancer · weekly oral monotherapy

ONCE WEEKLY. First 3 administrations 60 mg/m², total maximum 120 mg. From administration 4, 80 mg/m² (maximum 160 mg) only after the team reviews nadirs and toxicity. 60 or 50 mg/m²/lower may be needed. Continue weekly while helpful and tolerated; no fixed total number of weeks.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | Early treatment | Ongoing treatment | Dose change / pause | Next decision |
|---|---|---|---|---|---|
| **Treatment plan** NCCP 00259 | Confirm lung monotherapy plan and current dose | Weekly doses 1–3; review before dose 4 | Continue while helpful and tolerated | Review dose and recovery before restart | Agree continuation / next treatment |
| **Blood / kidney / liver** NCCP 00259 · SMPC | Baseline blood and organ tests | Counts before every weekly dose | Weekly treatment review | Review recovery and current dose | Follow unresolved abnormalities |
| **Symptoms / medicines** NCCP 00259 · SMPC | Bowel / nerve / infection review | Report early symptoms and support problems | Review all medicines and side effects | Do not restart or escalate yourself | Agree continuation and response assessment |
| **Special schedule check** NCCP 00259 · SMPC | Confirm weekly capsule plan | Review nadirs after doses 1–3 | Dose increases only after team review | Keep previous dose / nadir history | Personal plan; no booked dates implied |

# What each check is for

## 1 Treatment and dose review

Before every administration

Blood counts are checked on the day of each weekly dose. Kidney and liver tests follow the weekly regimen and prescribed plan. The team reviews the lowest counts after earlier doses before any increase, alongside bowel, nerve, infection and medicine checks.

**Ask / act:** The treating team confirms results, dose and suitability; this map does not authorise treatment.

Sources: NCCP 00259 · SMPC

## 2 Support and symptoms

Throughout treatment, including days at home

Use the treating unit’s 24-hour emergency number and local fever threshold. Contact immediately for fever/chills, sudden illness, breathlessness/chest pain, uncontrolled vomiting, severe abdominal pain/obstruction symptoms, bleeding or severe allergy. Do not wait until the next visit.

**Ask / act:** Keep a written medicine plan and urgent-contact route.

Sources: NCCP 00259 · ORAL-ANTIEMETIC · NCIS

## 3 Response and continuation

At agreed clinical review points

Vinorelbine chemotherapy can help control non-small-cell lung cancer and its symptoms. The team weighs possible benefit against side effects, previous treatments and your priorities, and reviews whether to continue.

**Ask / act:** Confirm the current plan with the team.

Sources: NCCP 00259 · TRIAL

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-02 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.29